

INDIVIDUAL CASE SAFETY REPORT (ICSR)

A Student-Prepared Pharmacovigilance Case Study

Report detail	Information
Report category	Educational ICSR simulation
Case reference	ICSR-EDU-001
Suspected medicine	Amoxicillin 500 mg
Suspected reaction	Skin rash and itching

Purpose: To demonstrate how a suspected adverse drug reaction is documented, assessed, and classified in an Individual Case Safety Report.

1. Introduction

An Individual Case Safety Report (ICSR) is a structured record of information about a suspected adverse drug reaction in one patient. It supports pharmacovigilance activities such as safety signal detection, medical review, follow-up, and regulatory reporting.

2. Aim

To prepare an educational ICSR by identifying the suspected adverse drug reaction, recording the medicine and patient information, assessing causality, and classifying seriousness and severity.

3. Case Description

Field	Case information
Patient	Patient A, 25-year-old male
Indication	Bacterial throat infection
Suspected drug	Amoxicillin 500 mg orally
Treatment start	Day 1
Reaction onset	Day 3 of treatment
Signs and symptoms	Red skin rash and itching over arms and chest
Action taken	Medicine stopped after medical advice
Clinical outcome	Symptoms gradually improved
Rechallenge	Not performed

4. Patient and Medical Information

The patient is a 25-year-old male receiving Amoxicillin for a bacterial throat infection. The case is fictional and prepared only for academic practice. In a real ICSR, the processor should collect relevant medical history, allergy history, concomitant medicines, laboratory findings, and reporter details.

5. Adverse Drug Reaction Identification

Suspected reaction: Skin rash and itching involving the arms and chest.

Reaction onset: Approximately two days after treatment initiation.

Course: Symptoms improved gradually after withdrawal of the suspected medicine.

Suggested coding concept: Skin rash / pruritus. The final MedDRA term must be selected by a trained case processor using the current terminology.

6. Suspected Drug Evaluation

Assessment item	Finding
Drug and dose	Amoxicillin 500 mg orally
Reason for use	Bacterial throat infection
Temporal relationship	Reaction appeared after drug initiation
Dechallenge	Positive; symptoms improved after withdrawal
Rechallenge	Not performed
Alternative causes	Not described in this fictional case

7. Causality Assessment

The relationship between Amoxicillin and the reaction is assessed as **Probable** for this educational case. The assessment is supported by the suitable time relationship, the compatibility of the symptoms with a known type of drug reaction, and improvement after withdrawal. A real case would require a complete medical review and assessment of alternative causes.

8. Seriousness, Severity and Expectedness

Parameter	Classification	Explanation
Severity	Mild	Rash and itching are described without disabling symptoms.
Seriousness	Non-serious	No death, life-threatening event, hospitalisation, disability, or other serious outcome is reported.
Expectedness	Possibly expected	The reaction is compatible with a known type of reaction; current product information must be checked.
Outcome	Improved	Symptoms gradually improved after withdrawal.

9. Follow-up Information Required

The following information should be requested before final case processing:

- Exact dates and times of the first dose and reaction onset.
- Previous penicillin or beta-lactam allergy history.
- All concomitant medicines and recent exposures.
- Any examination, laboratory findings, or treatment given.
- Final outcome and whether the reaction completely resolved.
- Reporter identity and contact information, where applicable.

10. Final Case Classification

Final field	Result
Suspected medicine	Amoxicillin 500 mg

Final field	Result
Suspected ADR	Skin rash and itching
Causality	Probable
Severity	Mild
Seriousness	Non-serious
Expectedness	Possibly expected / known type
Outcome	Improved after withdrawal

11. Conclusion

This educational ICSR demonstrates the basic pharmacovigilance process for documenting and assessing a suspected adverse drug reaction. In this fictional case, Amoxicillin is considered the probable suspected medicine, while the reaction is classified as mild and non-serious. A real report would require verification of the clinical information, coding, duplicate status, follow-up details, and applicable reporting requirements.

12. References

1. World Health Organization (WHO) resources on pharmacovigilance and adverse drug reaction monitoring.
2. ICH E2A and E2B principles for clinical safety data and Individual Case Safety Reports.
3. Standard textbooks of Pharmacology, Clinical Pharmacy, and Pharmacovigilance.
4. Current product information and MedDRA terminology guidance.

Academic note: This document is a fictional student simulation and is not a real patient safety report.